

1. Administrative & Study Identification

Full Study Title: A Study of Repotrectinib Versus Crizotinib in Participants With Locally Advanced or Metastatic Tyrosine Kinase Inhibitor (TKI)-naïve ROS1-positive Non-Small Cell Lung Cancer (NSCLC) (TRIDENT-3) **Short Title/Acronym:** TRIDENT-3 **Protocol Number:** CA127-1030 / U1111-1292-0487 **NCT Number:** NCT06140836 **Posting ID:** 69993455034349488 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Repotrectinib [Experimental] and Crizotinib [Active Comparator] **Phase of Development:** Phase 3 (PHASE3) **Medical Monitor:** Bristol Myers Squibb Clinical Operations / Medical Monitor

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of repotrectinib versus crizotinib as first-line therapy in participants with locally advanced or metastatic TKI-naïve ROS1-positive non-small cell lung cancer (NSCLC). **Secondary Objectives:** To evaluate Overall Response Rate (ORR), Overall Survival (OS), and intracranial response/efficacy.

2.2 Background & Rationale To address the need for improved first-line targeted therapies in ROS1-positive NSCLC. While first-generation TKIs like crizotinib are the current standard of care, they are limited by the eventual development of resistance and poor blood-brain barrier penetration, leading to central nervous system (CNS) progression. Repotrectinib is a next-generation ROS1/TRK TKI designed to overcome these resistance mutations and has demonstrated robust systemic and intracranial activity in earlier trials. TRIDENT-3 aims to confirm the superiority of repotrectinib over crizotinib in the TKI-naïve setting.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Crizotinib) **Blinding:** Open-label **Randomization:** Randomized (Parallel Group) **Trial Status:** Active, Not Recruiting (ACTIVE_NOT_RECRUITING)

3.2 Planned Enrollment & Duration Target \$N\$: 190 participants **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 21-Dec-2023 **Estimated Primary Completion:** 13-Mar-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participant has a histologically or cytologically confirmed diagnosis of locally advanced or metastatic NSCLC. 2. Participant has a ROS1 gene rearrangement/fusion as detected by a local test. 3. At least 1 measurable lesion according to RECIST v1.1, as assessed by the investigator. 4. Participants must not be exposed previously with TKIs that demonstrated activities in ROS1-positive NSCLC (TKI-naïve). Up to 1 prior line of systemic treatment for NSCLC is permitted. 5. ECOG Performance Status ≤ 2.

4.2 Exclusion Criteria 1. Symptomatic brain metastases or symptomatic leptomeningeal involvement. 2. History of previous cancer requiring therapy within the previous 2 years, except for NSCLC under study, squamous cell or basal-cell carcinoma of the skin, or any in situ carcinoma that has been completely resected. 3. Known tumor targetable co-mutations or rearrangements. 4. Clinically significant cardiovascular disease (either active or within 6 months prior to enrollment).

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: - *Arm A (Experimental)*: Repotrectinib administered orally. - *Arm B (Active Comparator)*: Crizotinib (Trade Name: Xalkori; ATC Code: L01ED01) administered orally. [EMA Product Information: [Xalkori EPAR](#)] **Route of Administration:** Oral. **Duration of Treatment:** Continuous treatment until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications for disease-related symptoms. **Prohibited:** Concurrent treatment with other investigational anticancer agents, prior ROS1-directed TKIs, or strong CYP3A inhibitors/inducers as defined by the protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, ROS1 Testing Confirmation, Baseline Imaging (CT/MRI), Medical History
Baseline	Day 0	Randomization, First Dose of Oral TKI, Baseline Safety Labs	Interim Every 8 Weeks Efficacy Assessment (Tumor Scans per RECIST v1.1), Safety Labs, AE Monitoring
End of Study	Follow-up	Discontinuation Visit, Final Vital Signs, Survival Follow-up	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Progression-free Survival (PFS) as per Blinded Independent Central Review (BICR) according to Response Evaluation Criteria in Solid Tumors (RECIST) v1.1.

Measurement Method: Central radiological review of CT/MRI scans.

7.2 Secondary & Exploratory Endpoints - Overall Survival (OS). - Overall Response Rate (ORR) as per BICR according to RECIST v1.1. - ORR as per Investigator according to RECIST v1.1.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine collection via patient interview, physical examinations, and clinical laboratory testing at all study visits. **Serious Adverse Events (SAEs):** Must be reported to the Sponsor/Pharmacovigilance team within 24 hours of site awareness. **Data Monitoring Committee (DMC):** An Independent Data Monitoring Committee (IDMC) will review safety and efficacy data periodically throughout the trial.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) Set for primary efficacy analysis (PFS); Safety Set for all patients receiving ≥ 1 dose of the study drug. **Sample Size Justification:** Target $N=190$ is powered to detect a statistically significant improvement in PFS for repotrectinib compared to crizotinib.

Handling of Missing Data: Censoring rules for PFS will follow standard FDA/EMA oncology guidelines for event-driven trials.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring visits to ensure protocol adherence and patient safety. **Audit Trail:** Validated Electronic Data Capture (EDC) system with full audit trail, eCRF locking, and rigorous source data verification.

11. Study Identifiers & Governance

Primary ID: CA127-1030 **Registry Number:** NCT06140836 **Posting ID:** 69993455034349488 **Sponsor/Lead Organization:** Bristol Myers Squibb
Study Title: TRIDENT-3 **Official Regulatory Title:** Randomized, Open-label, Multicenter, Phase 3 Trial of Repotrectinib Versus Crizotinib in Participants With Locally Advanced or Metastatic

12. Clinical Framework (Oncology Specific)

Primary Condition: {{Carcinoma, Non-Small-Cell Lung}} **Condition Name / Preferred Name:** {{Carcinoma}} **Preferred UMLS Name:** {{ROS1-Positive Non-Small Cell Lung Carcinoma}} **Semantic Type:** {{Neoplastic Process}} **Definition:** {{A malignant tumor arising from epithelial cells. Carcinomas that arise from glandular epithelium are called adenocarcinomas, those that arise from squamous epithelium are called squamous cell carcinomas, and those that arise from transitional epithelium are called transitional cell carcinomas (NCI Thesaurus).}} **Terminology Codes:** {{MeSH: D002277 | HPO: HP:0030731 | SNOMED: 1187425009}} **Diagnosis Threshold:** {{ROS1 gene rearrangement/fusion confirmed by local test}} **Medical Methodology:** {{Targeted Tyrosine Kinase Inhibitor (TKI) Therapy}} **Optimization Target:** {{Tumor response and progression-free survival}}

13. Study Procedures & Methodology

Management Pathway: {{Standard oncology pathway for advanced NSCLC}} **Education Protocol (HCP):** {{Investigator training on protocol and RECIST v1.1 evaluation}} **Education Protocol (Patient):** {{Informed consent process and oral TKI adherence education}} **Quality Audit Mechanism:** {{Blinded Independent Central Review (BICR) of imaging and clinical source data verification}}

14. Observation & Follow-Up Schedule

Total Duration: {{Up to approximately 41 months of follow-up for PFS}} **Onsite Visit Cadence:** {{Every 8 weeks for tumor assessment and clinical evaluation}} **Remote Monitoring Frequency:** {{Continuous safety and AE monitoring throughout the treatment phase}} **Checklist Review Interval:** {{Every 8 weeks (Q8W) concurrent with RECIST v1.1 scans}}

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				